

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient, at 32 weeks gestation, has a medium risk for preeclampsia and macrosomia. Key findings include an elevated systolic blood pressure of 142 mmHg and proteinuria (dipstick 1), which contribute to the medium preeclampsia risk. For macrosomia risk, the patient's high pre-pregnancy BMI of 42.4 and a fasting glucose of 99 mg/dL are noted. Given the medium preeclampsia risk, an alert to the doctor is recommended for further evaluation and management.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED, 32 weeks gestation, presents with a medium preeclampsia risk (score: 0.6395) and a medium macrosomia risk (score: 0.5944). Key clinical findings include systolic BP 142 mmHg, diastolic BP 79 mmHg, and proteinuria dipstick 1. No severe headache, vision changes, upper abdominal pain, or swelling of face/hands reported. Fasting glucose is 99 mg/dL. Maternal age 38, pre-pregnancy BMI 42.4. Per ACOG guidance on hypertensive disorders of pregnancy, these findings warrant further clinical assessment. Recommend immediate physician review for evaluation and management.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.